



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سافيناز محمدى حسين نوار

العنوان : عماره 4 م أ مدينه
الجامعات

الحى / المدينه : بنها

المحافظة : القليوبيه

تاريخ تسجيل الإستماره : 23-09-2026

رقم البطاقة : 25902060102161 رقم التليفون : 01289307222

العمر(تاريخ الميلاد) : 06-02-1959 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
التوقيع: (-----)

الإحصائي الاجتماعي
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الاورام

خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي
الاورام المنتشر [14456]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سافيناز محمدى حسين نوار National ID: 25902060102161	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

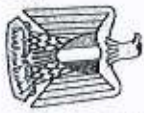
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pin Medication And Anti Emetics
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


 جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 مافيل
 محمدي حسين نوار
 صادره - تم اصداره الجامعة القديس
 بنها - القليوبية
 ٢٥٩٠٢٠٦٠١٠٢١٦١
 ١٨٥٩١٠٢١٦١
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 ٢٠٢٢/١١ ٢٥٩٠٢٠٦٠١٠٢١٦١
 ربه منزل
 انش
 احمد محمد رضا العلي
 البطاقة سارية حتى ٢٠٢٩/١١/٢٥
 ١١



03/09/2026

MDT FORM

Name:	سليمان محمدى حسن لوتى	MRN:	9159
Age:	67 Y	ID:	25902060102161
Menopausal Sta		Phone:	01289307222
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX:

ER: 8/8 PR: 8/8 HER2: 2+ sish n KI67:

R

L

Sonomammography:

Mets Workup:

met breast to bone bilat pulm

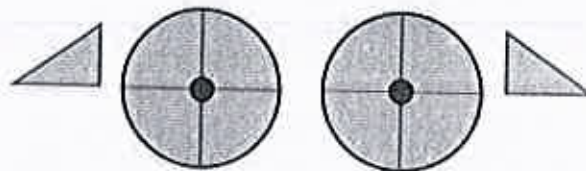
History:

Recommendations:

for AI ZOMETA

MDT Outcome & Plan

for AI ZOMETA



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نوره مهنى	د/ خالد المهنى				

Patient Name	: الأستاذ / سافيناز محمدي حسين	Request No.	: 13005074
Patient No.	: 25902060102161	Request Date	: 16/02/2026
Path No.	: 461/26	Verification Date	: 22/02/2026
Sex	: Female	Financial Category	: جوستاف روسي - اقتصادي
Age	: 67 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass (BIRADs 5), diagnosed by (H&E) as invasive duct carcinoma in a request No (13005074).

Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- P63 clone (4A4), ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

* IPT (Positive control showed appropriate reactivity):

- P63: Negative.
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- Ki67%: 50%.

Conclusion:

Left breast mass, Tru-cut biopsy:

IPT:

- P63: Negative.
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+).
- * Case for further (SISH). →
- Ki67%: 50%.

* Scoring system: "ER, PR"

Verified by :

Dr.Duaa Mohamed

Printing Date : 24/02/2026 11:42:57

Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	الأستاذة / سافيناز محمدي حسين	Request No.	13005074
Patient No.	25902060102161	Request Date	16/02/2026
Path No.	461/26	Verification Date	22/02/2026
Sex	Female	Financial Category	جوستاف روسي - اقتصادي
Age	67 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / سافيناز محمدي حسين
Patient No. : 25902060102161
Path No. : 461/26
Sex : Female
Age : 67 Y
Referring Doc :

Request No. : 13005074
Request Date : 16/02/2026
Verification Date : 21/02/2026
Financial Category : جوستاف روسي - اقتصادي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical data

Left breast mass.

Radiology results:

-Highly suspicious looking mass(an irregular shaped mass with speculated margin and multiple foci grouped microcalcifications inside is seen in the lower upper quadrant) associated with an adjacent area of distortion BIRADs 5.

Nature of specimen

Tru cut biopsy, collected on 3:00 pm, cold ischemia time is 10 sconds.

Gross examination

Multiple (4) grayish white tissue cores raning in size from 0.8 up to 1.2 cm, totally submitted.

Microscopic examination

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+3+2) Grade 2
- * DCIS: Absent.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.

Diagnosis

Left breast mass, Tru cut biopsy:

Invasive duct carcinoma, Grade 2.

For further IHC (ER, PR, HER2, Ki67)

Signature

Prof. Dr. Gamal Abdel Aty
Dr.Menan Hosam

Verified by :

Dr.Heba Hasan

Printing Date : 21/02/2026 13:26:37

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- Unremarkable posterior fossa structures.

B. Neck:

- FDG avid left deep cervical LN is noted, measuring about 1.6x1.3 cm with SUVmax~6.1 likely metastatic.
- No FDG avid thyroid nodules.
- Normal appearance and physiologic FDG uptake of the nasopharynx, oropharynx and laryngeal air spaces.
- Normal appearance and physiologic FDG uptake of submandibular and parotid salivary glands.

C. Chest:

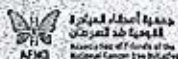
- The left breast showing mild diffuse skin thickening with FDG soft tissue lesions with lobulated outline at its UOQ reaching the skin, measuring roughly about 8.1x8.5x7.2 cm along maximum axial & CC dimensions with SUVmax~20.8.
- Few FDG avid left axillary & sub-pectoral LNs are noted, with largest seen measuring about 1.7x1.3 cm with SUVmax~5.4.
- No FDG avid right breast lesions or right axillary LNs.
- FDG avid few bilateral scattered pulmonary nodules "likely metastatic" are noted, with largest seen at right upper lung lobe measuring about 2.3x1.7 cm with SUVmax~12.3.
- FDG avid mildly enlarged pre-vascular LN is noted "likely metastatic", measuring about 1.9x1.4 cm with SUVmax~20.8
- No evidence of pleural effusion, pleural thickening or masses seen.
- No pericardial effusion, thickening or masses detected.
- Normal physiologic FDG uptake of the heart.

D. Abdomen and Pelvis:

- No hyper-metabolic abdominal or pelvic lymphadenopathy.
- Enlarged liver with no FDG avid focal lesions.

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فاكس : 25 35 1745 (202)
57357Egypt

9109

Name: Mrs. Safinaz Mohammady Hussein

Date: 18/02/2026

ID: 30207191

Whole Body 18F-FDG PET/CT

History and clinical findings:

67-year-old female patient with newly diagnosed left breast IDC.

Reason for the study: Initial PET/ CT assessment.

Technique:

Radio-pharmaceutical: F-18-FDG, MBq intra-venous infusion.

Imaging sequence:

- * Approximately 60 minutes following radiopharmaceutical administration, PET-CT was acquired from mid-thigh to the skull vault with the arms extended above the head.
- * Qualitative and semi-quantitative assessment of the PET images was done.
- * Diagnostic CT was acquired in addition to the purpose of anatomic localization and attenuation correction.
- * IV contrast was administered.
- * Data was processed and displayed according to departmental protocol.

FINDINGS:

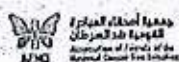
A. Brain:

- No gross hyper-metabolic brain SOLs.
- No gross focal cerebral or cerebellar area of abnormal density seen.
- No gross extra-axial or intra-cerebral areas of fresh blood densities.
- No midline shift.

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- Physiologic diffuse FDG uptake is seen over average size spleen with no FDG avid focal lesions seen.
- Normal appearance of both kidneys with no back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.
- *Prominent left adrenal gland.* Yet no active adrenal nodules.
- Normal appearance of the pancreas, aorta & I.V.C with no FDG avid lesions detected.
- *Mesenteric panniculitis noted.*
- No FDG avid uterine or adnexal lesions.
- No evidence of ascites or abdominal collections noted.

E. Musculoskeletal:

- FDG avid multiple lytic & sclerotic metastatic osseous lesions are seen involving axial & appendicular skeleton dominant at the spine and pelvic bones showing variable grades of FDG activity of SUVmax~10.9.
- *Diffuse spondylo-degenerative changes are noted.*
- *1st degree degenerative spondylolisthesis of L4 over L5.*
- *Bilateral sacroiliitis, more on the left.*

الطبيب
د. رشا أنوار
مستشفى
السرطان - 1991

Nuclear Medicine Dept. PET/CT Unit

Conclusion:

Initial FDG PET/CT study revealed:

- Left breast FDG soft tissue lesions with lobulated outline at its UOQ reaching the skin, (site of primary neoplasm).
- Few FDG avid left lower cervical, mediastinal prevascular, left axillary & sub-pectoral LNs, (metastatic deposits).
- FDG avid few bilateral scattered pulmonary nodules "likely metastatic".
- Wide spread FDG avid metastatic osseous deposits.

Thanks for referral;

Dr. Rasha Anwar, MD.

Consultant of Radio-diagnosis.

Dr Esraa El-Kholy, MD

Consultant of Nuclear Medicine, CCHE
Professor of Nuclear Medicine, NCI
Cairo University

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DMS- 9/59

Dr. SAFINAZ MOHAMEDYOUSSEIN

Date : 16-07-2026 01:10

Unit : Portable

Medical No. : GRI-160226010852PM

Physician Prof :

Ins. Company :

MATERIAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Collection: Palpable left breast mass.

- **Breast density:** Predominantly fatty breast parenchyma...Matching ACR a.
- An irregular shaped mass with speculated margin and multiple foci grouped micro-calcifications inside is seen in the lower upper quadrant of the left breast, associated with adjacent area of architectural distortion retracting the nipple
- No detected suspicious mass density, architectural distortion or pathological micro-calcification in the right breast.

By complementary breast ultrasound:

- Homogenous (predominantly fatty) echo-texture of both breasts.
- An irregular shaped non circumscribed margin hypoechoic mass is seen in the left breast at 1-2 o'clock position. It measures (8.5 x 6.8 x 7.7 cm) reaching the overlying skin, as well as internal vascularity detected within.
- No detected suspicious solid or cystic masses in the right breast.
- Few indeterminate left axillary lymph nodes, largest measures (2.4 x 1.9 cm) with cortical thickness of 4.3 mm
- Pathological left infraclavicular lymph node with globular shape and lost cortical hilum measuring 15 x 12 mm.
- Right indeterminate axillary lymph node measuring 20 x 8 mm with cortical thickness of 3.5 mm.

CONCLUSION:

- Highly suspicious looking left breast mass associated with an adjacent small area of distortion BI-RADS 5 category.

Dr farah elshobaky

